CCDM - Criticality-Coded Delegation Matrix

Site-defined task requirements, delegation boundaries and supervision

SIMULATED · v0.2

Study / protocol GLENOS-01 / GS-2026-01	**Site / code** Riverside Orthopedic Center / 014	**CCDM record ID** CCDM-014	**Record revision** v0.2
Applicable CIP / protocol version CIP v4.2	**Task / process scope** Surgery, device, imaging, follow-up, safety and data	**Matrix owner / role** Maria Silva / Study Coordinator	**Review date** 29 Sep 2026
Review basis / type Triggered review: PD-014-07	**Approval status** ☐ Approved ☑ With restrictions ☐ Not approved	**Effective date** 30 Sep 2026	**Next scheduled review** 15 Dec 2026
Supersedes - record ID / revision CCDM-014 v0.1 / 15 Sep 2026		**Change trigger / affected scope** Unplanned glenoid baseplate substitution not escalated: review intraoperative supervision and device routing.	
Controlled record location / included matrix sheets ISF / Delegation / CCDM-014 / v0.2; 1 matrix sheet.			

A Purpose and Use - site-defined delegation matrix

Purpose and Use: Use one controlled CCDM for a defined site, study and revision. The site identifies its tasks and decisions, assigns criticality and supervision, and records qualifications, permitted boundaries and escalation triggers for approval. The approved matrix defines the task requirements applied in training requirements, person-task assignment and release records. It supports, but does not replace, PI oversight, risk assessment, individual competency evidence or task release.

Element	Complete when	Record / link
Unit of analysis	One controlled matrix for one defined site, study and revision; one row in F for each task or decision.	Record ID, revision, scope and current task inventory.
Decision supported	Approve which tasks are retained or delegable, with the required qualification, supervision, boundaries and escalation.	Site-completed F rows, approval status and any restrictions.
Inputs	Current CIP/protocol, site roles, risk controls, sponsor requirements and applicable procedures.	Controlled IDs/versions for POT, SRR and relevant specialist records.
Outputs	An approved task reference for training requirements, person-task assignment and release, with traceable revision control.	Feeds DTRM (training per task), RACI (who takes each task, using these codes), TRE (requirements checked at release) and POT (what requires the PI).

B Scope boundary and interfaces

Control / interface	What it defines or evaluates	Relationship to CCDM
RACI	Who is responsible, accountable, consulted and informed for each task, with role holder, backup and acceptance.	CCDM sets how critical each task is and how much supervision it needs; RACI assigns people using those codes.
DTRM / TRE	Training each role or task requires, and the release of each person.	CCDM defines task requirements; it does not demonstrate individual competence or release a person.
SRR	Site risk identification, assessment and control follow-up.	CCDM records the resulting task boundaries and supervision; it does not replace risk assessment.
POT / PI oversight	PI accountability, review arrangements and final decision boundaries.	CCDM does not transfer PI accountability to delegated staff.
Specialist records	SCR, ICDL, SIRR/IQL, SLCT or other records relevant to the site's task scope.	CCDM references applicable qualifications and controls; it does not replace their evidence.

C Criticality code definitions

Code	Decision boundary	Delegability	Allowed supervision levels	PI trigger
C1	PI-final decision or accountability cannot be transferred.	**Not delegable.** Staff may prepare information or perform administrative support only.	**S0** only.	Any ambiguity, exception, safety concern or disagreement.
C2	Material clinical, procedural, device or endpoint activity requiring active PI review.	Delegable only within a defined boundary after task-specific release.	**S1** or **S2**.	Out-of-boundary case, complication, deviation, unexpected result or recurring concern.
C3	Routine operational activity with no independent PI-final decision.	Delegable after qualification, current training and demonstrated competence.	**S2** or **S3**.	Uncertainty, repeated failure, missed window, participant concern or data-quality signal.

D Supervision levels

Level	Name	Operational meaning
S0	**PI-retained**	The delegated person may support preparation but may not perform the final decision or act independently within the task.
S1	**Active PI review**	The person may perform defined steps, but the PI or named specialist reviews specified outputs before or at the defined decision point.
S2	**Defined supervision**	The person may perform the task after release, with a named supervisor, stated restrictions and case-level escalation triggers.
S3	**Routine oversight**	The person may perform the task independently within current instructions; performance is reviewed periodically through normal quality controls.

E Matrix review and approval

Review the matrix when protocol, procedures, devices, risks, task scope, PI/supervision coverage or relevant quality signals change. The owner assesses affected rows and release records. The PI or accountable approver authorizes the matrix and any restrictions before its effective date.

Approval check	Yes	No	Evidence / action / restriction
All tasks have CtQ domain, code, supervision, qualifications, boundaries, escalation and record links.	☑	☐	Section F: T01-T10; 2 C1 / 6 C2 / 2 C3.
Task requirements agree with current protocol, risk controls, oversight and applicable procedures.	☑	☐	CIP v4.2; POT v0.2; SRR v0.3.
RACI assignments and TRE release records will reference this approved CCDM record ID and revision.	☑	☐	RACI-014 updated 29 Sep; linked TRE records checked.
The review date, change triggers, controlled location and any unresolved restriction are documented.	☑	☐	Review 15 Dec; restriction: first cases supervised; location on page 1.

Matrix owner

Name / role: Maria Silva / Study Coordinator

Signature / date: /s/ M. Silva - 29 Sep 2026 (demo)

PI / accountable approver

Name / role: Dr Alex Morgan / PI

Signature / date: /s/ A. Morgan - 29 Sep 2026 (demo)

CCDM - Criticality-Coded Delegation Matrix

Site-completed task matrix / continuation sheet

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Study / site	CCDM record ID / revision	Effective date	Matrix sheet number / total sheets
GLENOS-01 / Site 014	CCDM-014 / v0.2	30 Sep 2026	1 / 1

F Controlled task, criticality and supervision matrix

Use one row per task or decision, with a task ID (T01, T02...) that the DTRM, TRE and RACI cite. Separate activities with different decision boundaries; for inseparable activities, apply the highest relevant criticality. Define the required supervision and escalation for each row.

Task ID	Task / decision	CtQ domain	Code	Supervision level	Minimum role / qualification	Permitted boundary	PI / specialist escalation trigger	Linked controlled record
T01	Final eligibility decision	CtQ2 Participant selection	C1	S0	PI; protocol and consent training current.	Staff prepare PSEAR; PI decides before enrollment. No protocol waivers.	Borderline imaging, rotator cuff status or consent doubt: pause enrollment; refer to PI.	PSEAR; CIP v4.2; POT
T02	Final clinical safety classification and causality	CtQ10 Safety surveillance	C1	S0	PI; safety procedure training current.	Staff prepare SART and route reports; PI retains seriousness and device-relatedness assessment.	Serious, unexpected or device-related event: PI and sponsor safety contact.	SART; POT
T03	Reverse shoulder arthroplasty implantation per surgical technique	CtQ5 Procedural delivery	C2	S1 + S2	Credentialed shoulder surgeon; SCR and device TRE release current.	Released surgeon follows the approved technique; senior shoulder surgeon supervises the first cases.	Unplanned component substitution, glenoid fixation problem or intraoperative fracture: protect participant, document, notify PI.	SCR; TRE; ICDL; SLCT
T04	Implant identification (UDI, lot, size) and component substitution record	CtQ4 Device traceability	C2	S2	OR nurse; ICDL and device training current.	Record each component before closure; record any substitution with its reason.	Missing or unreadable label, substitution without reason: hold the case record; notify surgeon and PI.	ICDL; TRE
T05	Postoperative radiographs per imaging charter (true AP, axillary)	CtQ9 Imaging and endpoint control	C2	S1 + S2	Radiographer; imaging charter training and TRE release current.	Acquire the charter set; radiology liaison checks quality before transfer.	Non-diagnostic view, wrong projection or outside window: repeat while possible; radiology liaison and PI.	SIRR; IQL; TRE
T06	ROM measurement and PROM administration (Constant-Murley, ASES)	CtQ8 Endpoint assessment	C2	S2	Functional assessor (physiotherapist); assessment method training and TRE release current.	Measure with the calibrated goniometer by the defined method; administer PROMs without interpreting them.	Assessor change, missed method step or participant unable to complete: coordinator and PI.	PFVR; TRE
T07	Follow-up scheduling and window tracking	CtQ7 Care pathway and follow-up	C3	S3	Study coordinator; visit-window training current.	Schedule 6-week, 3-month and 12-month visits within window; record contacts; route symptoms to clinicians.	Missed window, failed contact or participant concern: PI; recovery action.	PFVR; FUCR; CIP v4.2
T08	eCRF transcription and controlled data filing	CtQ11 Data capture and source	C3	S3	Data QA function; EDC and document training and access current.	Transcribe verified sources; file approved records; clinically meaningful discrepancies go to the PI.	Conflicting source, wrong version or unresolved clinical query: data lead and PI.	EDC audit trail; ISF
T09	Device accountability, returns and explant handling	CtQ4 Device traceability	C2	S1 + S2	Device custodian; accountability training and TRE release current.	Reconcile against ICDL; senior shoulder surgeon reviews discrepancies and explants.	Missing device, discrepancy, explant or custody gap: custodian, PI and sponsor device route.	ICDL; sponsor accountability log
T10	Safety intake and initial reporting route	CtQ10 Safety surveillance	C2	S2	Study coordinator and trained deputy; safety procedure training current.	Capture awareness time; alert PI; route reports before final classification.	Uncertain seriousness, device relationship or deadline: immediate PI and sponsor contact.	SART; on-call rota

If more tasks are needed, reprint this page, number each matrix sheet and include all sheets in the final approved CCDM.